

The adult programme, for specialists.

A multidisciplinary medical–psychological intervention for disorders of gut–brain interaction — indications, rationale, and value to your practice.

Rose Carson · Dr Sarah Anticich · Dr Nicholas Anticich (selected workshops)

Overview

A structured, evidence-informed group intervention for adults with disorders of gut–brain interaction. The programme integrates gastroenterological science, clinical psychology and behavioural medicine to address the biological, emotional and cognitive mechanisms that sustain chronic gastrointestinal symptoms. It complements specialist medical care, and is particularly suited to patients who remain functionally impaired despite appropriate investigation and treatment.

Indications for referral

- IBS, functional dyspepsia, centrally mediated abdominal pain, motility disorders.
- Stress-sensitive or autonomically driven GI symptoms.
- Persistent symptoms despite normal or reassuring investigations.
- High health anxiety, symptom hypervigilance, or repeated reassurance-seeking.

Referral is appropriate at any stage of the diagnostic pathway — early presentation, post-investigation, or long-standing and complex cases.

Clinical rationale

DGBIs reflect neurophysiological dysregulation of the gut–brain axis rather than structural pathology alone. Symptom severity and persistence are shaped by autonomic activation, emotional reactivity and stress sensitivity, cognitive threat appraisal, executive functioning overload, and disrupted behavioural rhythms. Care models focused solely on diet, medication or reassurance often fail to address these sustaining drivers — this programme fills that gap through coordinated medical–psychological intervention.

VALUE TO YOUR PRACTICE

Patients entering or returning to gastroenterology care are typically better regulated, more informed, and more engaged — supporting improved efficiency and clinician satisfaction.

- Reduces repeat reassurance-based consultations and low-yield investigations.
- Improves patient regulation, understanding, and readiness for review.
- Allows more efficient use of specialist consultation time.
- Generalises specialist knowledge and care principles across larger cohorts.

Positioning: the programme does not replace medical management — it bridges the gap between investigation and functional recovery, supports patients who are medically stable but symptomatically distressed, and enhances outcomes through coordinated multidisciplinary care.

Model of care — The Executive Frame™

Emotional intelligence

Regulation of emotional and physiological states, reducing sympathetic overactivation and stress-related gut reactivity.

Executive functioning

Planning, pacing, attentional control, task initiation and habit formation — consistency without overwhelm.

Self-determination

Autonomy, competence and connection — the drivers of motivation, adherence and long-term behaviour change.

Programme content

- Contemporary gut–brain and neurogastroenterology education.
- Emotional regulation and nervous system stabilisation strategies.
- Cognitive flexibility and symptom reframing.
- Behavioural medicine and micro-behavioural interventions.
- Values-based goal setting and sustainable microsteps.

REFERRAL & DELIVERY	
Format	Half-day workshops, face-to-face · Auckland and Christchurch · adult cohorts
Suits	Early triage, waitlist support, or entry point into longer programmes
Cost	\$350 per person
Christchurch	Saturday 21 February · 9:00am–12:00pm
Auckland	Tuesday 3 February · 1:00pm–5:00pm
Referral	Direct referral · confirmed diagnosis not required · rosec@macmurray.co.nz